

ADVERTISEMENT

loperamide (Rx, OTC)

Brand and Other Names: Imodium, K-Pek II, NeoDiaral, Diaraid**Classes:** Antidiarrheals

Dosing & Uses

ADULT

Dosage Forms & Strengths

liquid

- 1mg/7.5mL

suspension/solution

- 1mg/7.5mL

tablet

- 2mg

capsule

- 2mg

Acute Diarrhea

4 mg initially, then 2 mg after each loose stool; not to exceed 16 mg/day (8 mg/day for self-medication); discontinue if no improvement seen within 48 hours

Chronic Diarrhea

4 mg initially, then 2 mg after each loose stool until controlled, and then 4-8 mg/day in divided doses

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Traveler's Diarrhea

4 mg after first loose stool, then 2 mg after each subsequent stool; not to exceed 8 mg/day

PEDIATRIC**Dosage Forms & Strengths****liquid**

- 1mg/7.5mL

suspension/solution

- 1mg7.5mL

tablet

- 2mg

capsule

- 2mg

Acute Diarrhea**First Day of Treatment**

- 2-6 years (13-20 kg): 1 mg q8hr PO
- 6-8 years: (20-30 kg): 2 mg q12hr PO
- 8-12 years (>30 kg): 2 mg q8hr PO

Second & Subsequent Doses

- 0.1 mg/kg PO after each loose stool; not to exceed dose recommended for first 24 hours

Chronic Diarrhea

0.08-0.24 mg/kg/day PO divided q12hr

Traveler's Diarrhea

<6 years: Safety and efficacy not established

6-8 years: 2 mg after first loose stool, then 1 mg after each subsequent stool; not to exceed 4 mg/day

8-12 years: 2 mg after first loose stool, then 1 mg after each subsequent stool; not to exceed 6 mg/day

>12 years: 4 mg after first loose stool, then 2 mg after each subsequent stool; not to exceed 8 mg/day

Interactions

Interaction Checker

Enter a drug name to check for any interactions. and loperamide

No Interactions Found

Interactions Found

Contraindicated

Serious

Significant - Monitor Closely

Minor

All Interactions Sort By:

Contraindicated (0)

Serious (37)

- amisulpride
amisulpride and loperamide both increase QTc interval. Avoid or Use Alternate Drug. ECG monitoring is recommended if coadministered.
- anagrelide
anagrelide and loperamide both increase QTc interval. Avoid or Use Alternate Drug.
- artemether
artemether and loperamide both increase QTc interval. Avoid or Use Alternate Drug.
- artemether/lumefantrine
artemether/lumefantrine and loperamide both increase QTc interval. Avoid or Use Alternate Drug.
- asenapine
asenapine and loperamide both increase QTc interval. Avoid or Use Alternate Drug.

- asenapine transdermal
asenapine transdermal and loperamide both increase QTc interval. Avoid or Use Alternate Drug.
- bedaquiline
bedaquiline and loperamide both increase QTc interval. Avoid or Use Alternate Drug.
- buprenorphine
buprenorphine and loperamide both increase QTc interval. Avoid or Use Alternate Drug.
- buprenorphine buccal
buprenorphine buccal and loperamide both increase QTc interval. Avoid or Use Alternate Drug.
- buprenorphine subdermal implant
buprenorphine subdermal implant and loperamide both increase QTc interval. Avoid or Use Alternate Drug.
- buprenorphine transdermal
buprenorphine transdermal and loperamide both increase QTc interval. Avoid or Use Alternate Drug.
- buprenorphine, long-acting injection
buprenorphine, long-acting injection and loperamide both increase QTc interval. Avoid or Use Alternate Drug.
- ceritinib
ceritinib and loperamide both increase QTc interval. Avoid or Use Alternate Drug.
- chloroquine
chloroquine and loperamide both increase QTc interval. Avoid or Use Alternate Drug.
- citalopram
citalopram and loperamide both increase QTc interval. Avoid or Use Alternate Drug.
- clarithromycin
clarithromycin and loperamide both increase QTc interval. Avoid or Use Alternate Drug.
- clozapine
clozapine and loperamide both increase QTc interval. Avoid or Use Alternate Drug.
- crizotinib
crizotinib and loperamide both increase QTc interval. Avoid or Use Alternate Drug.
- desflurane
desflurane and loperamide both increase QTc interval. Avoid or Use Alternate Drug.
- eluxadoline

loperamide, eluxadoline. Either increases effects of the other by pharmacodynamic synergism. Avoid or Use Alternate Drug. Avoid coadministration with other drugs that cause constipation. Increases risk for constipation related serious adverse reactions. Loperamide may be used occasionally for acute management of severe diarrhea but avoid chronic use. Discontinue loperamide immediately if constipation occurs.

- **entrectinib**
entrectinib and loperamide both increase QTc interval. Avoid or Use Alternate Drug.
- **erdafitinib**
erdafitinib will increase the level or effect of loperamide by P-glycoprotein (MDR1) efflux transporter. Avoid or Use Alternate Drug. If coadministration unavoidable, separate administration by at least 6 hr before or after administration of P-gp substrates with narrow therapeutic index.
- **eribulin**
eribulin and loperamide both increase QTc interval. Avoid or Use Alternate Drug.
- **fentanyl**
fentanyl, loperamide. Either increases effects of the other by pharmacodynamic synergism. Avoid or Use Alternate Drug. Coadministration with other CNS depressants, such as skeletal muscle relaxants, may cause respiratory depression, hypotension, profound sedation, coma, and/or death. Consider dose reduction of either or both agents to avoid serious adverse effects. Monitor for hypotension, respiratory depression, and profound sedation.
- **fentanyl intranasal**
fentanyl intranasal, loperamide. Either increases effects of the other by pharmacodynamic synergism. Avoid or Use Alternate Drug. Coadministration with other CNS depressants, such as skeletal muscle relaxants, may cause respiratory depression, hypotension, profound sedation, coma, and/or death. Consider dose reduction of either or both agents to avoid serious adverse effects. Monitor for hypotension, respiratory depression, and profound sedation.
- **fentanyl transdermal**
fentanyl transdermal, loperamide. Either increases effects of the other by pharmacodynamic synergism. Avoid or Use Alternate Drug. Coadministration with other CNS depressants, such as skeletal muscle relaxants, may cause respiratory depression, hypotension, profound sedation, coma, and/or death. Consider dose reduction of either or both agents to avoid serious adverse effects. Monitor for hypotension, respiratory depression, and profound sedation.
- **fentanyl transmucosal**
fentanyl transmucosal, loperamide. Either increases effects of the other by pharmacodynamic synergism. Avoid or Use Alternate Drug. Coadministration with other CNS depressants, such as skeletal muscle relaxants, may cause respiratory depression, hypotension, profound sedation, coma, and/or death. Consider dose reduction of either or both agents to avoid serious adverse effects. Monitor for hypotension, respiratory depression, and profound sedation.
- **fexinidazole**

fexinidazole and loperamide both increase QTc interval. Avoid or Use Alternate Drug. Avoid coadministration of fexinidazole with drugs known to block potassium channels or prolong QT interval.

- givinostat
loperamide and givinostat both increase QTc interval. Avoid or Use Alternate Drug. If unable to avoid coadministration, obtain ECGs when initiating, during concomitant use, and as clinically indicated. Withhold if QTc interval >500 ms or a change from baseline >60 ms.
- isoflurane
isoflurane and loperamide both increase QTc interval. Avoid or Use Alternate Drug.
- lefamulin
lefamulin and loperamide both increase QTc interval. Avoid or Use Alternate Drug.
- oxaliplatin
oxaliplatin and loperamide both increase QTc interval. Avoid or Use Alternate Drug.
- quinidine
quinidine will increase the level or effect of loperamide by P-glycoprotein (MDR1) efflux transporter. Avoid or Use Alternate Drug.
- sevoflurane
sevoflurane and loperamide both increase QTc interval. Avoid or Use Alternate Drug.
- sotorasib
sotorasib will decrease the level or effect of loperamide by P-glycoprotein (MDR1) efflux transporter. Avoid or Use Alternate Drug. If use is unavoidable, refer to the prescribing information of the P-gp substrate for dosage modifications.
- tepotinib
tepotinib will increase the level or effect of loperamide by P-glycoprotein (MDR1) efflux transporter. Avoid or Use Alternate Drug. If concomitant use unavoidable, reduce the P-gp substrate dosage if recommended in its approved product labeling.
- tetrabenazine
tetrabenazine and loperamide both increase QTc interval. Avoid or Use Alternate Drug.

Monitor Closely (91)

- alfuzosin
alfuzosin and loperamide both increase QTc interval. Use Caution/Monitor.
- amiodarone
amiodarone will increase the level or effect of loperamide by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.
- apomorphine

apomorphine and loperamide both increase QTc interval. Use Caution/Monitor.

- aripiprazole
aripiprazole and loperamide both increase QTc interval. Use Caution/Monitor.
- atomoxetine
atomoxetine and loperamide both increase QTc interval. Use Caution/Monitor.
- atorvastatin
atorvastatin will increase the level or effect of loperamide by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.
- beralstat
beralstat will increase the level or effect of loperamide by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor. Monitor or titrate P-gp substrate dose if coadministered.
- bosutinib
bosutinib increases levels of loperamide by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.
- clarithromycin
clarithromycin will increase the level or effect of loperamide by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.
- clotrimazole
clotrimazole will decrease the level or effect of loperamide by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.
- crizotinib
crizotinib increases levels of loperamide by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.
- cyclosporine
cyclosporine will increase the level or effect of loperamide by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.
- danicopan
danicopan will increase the level or effect of loperamide by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor. Danicopan increases plasma concentrations of P-gp substrates; consider dose reduction of P-gp substrates where minimal concentration changes may lead to serious adverse reactions.
- dasatinib
dasatinib and loperamide both increase QTc interval. Use Caution/Monitor.
- degarelix
degarelix and loperamide both increase QTc interval. Use Caution/Monitor.
- deutetrabenazine
deutetrabenazine and loperamide both increase QTc interval. Use Caution/Monitor. At the maximum recommended dose, deutetrabenazine does not prolong QT interval to a clinically relevant extent. Certain circumstances may increase risk of

torsade de pointes and/or sudden death in association with drugs that prolong the QTc interval (eg, bradycardia, hypokalemia or hypomagnesemia, coadministration with other drugs that prolong QTc interval, presence of congenital QT prolongation).

- dolasetron
dolasetron and loperamide both increase QTc interval. Use Caution/Monitor.
- donepezil
donepezil and loperamide both increase QTc interval. Use Caution/Monitor.
- dordaviprone
dordaviprone and loperamide both increase QTc interval. Use Caution/Monitor.
Increase the frequency of ECG and electrolyte monitoring.
- doxepin
doxepin and loperamide both increase QTc interval. Use Caution/Monitor.
- dronedarone
dronedarone will increase the level or effect of loperamide by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.
- efavirenz
efavirenz and loperamide both increase QTc interval. Use Caution/Monitor.
- elagolix
elagolix will increase the level or effect of loperamide by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.
- eliglustat
eliglustat increases levels of loperamide by P-glycoprotein (MDR1) efflux transporter. Modify Therapy/Monitor Closely. Monitor therapeutic drug concentrations, as indicated, or consider reducing the dosage of the P-gp substrate and titrate to clinical effect.

eliglustat and loperamide both increase QTc interval. Use Caution/Monitor.
- erythromycin base
erythromycin base will increase the level or effect of loperamide by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.
- erythromycin ethylsuccinate
erythromycin ethylsuccinate will increase the level or effect of loperamide by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.
- erythromycin lactobionate
erythromycin lactobionate will increase the level or effect of loperamide by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.
- erythromycin stearate
erythromycin stearate will increase the level or effect of loperamide by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.

- felodipine
felodipine will increase the level or effect of loperamide by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.
- fosphenytoin
fosphenytoin will decrease the level or effect of loperamide by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.
- fostamatinib
fostamatinib will increase the level or effect of loperamide by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor. Concomitant use of fostamatinib may increase concentrations of P-gp substrates. Monitor for toxicities of the P-gp substrate drug that may require dosage reduction when given concurrently with fostamatinib.
- fostemsavir
loperamide and fostemsavir both increase QTc interval. Use Caution/Monitor. QTc prolongation reported with higher than recommended doses of fostemsavir.
- gemifloxacin
gemifloxacin and loperamide both increase QTc interval. Use Caution/Monitor.
- gilteritinib
gilteritinib and loperamide both increase QTc interval. Use Caution/Monitor.
- glecaprevir/pibrentasvir
glecaprevir/pibrentasvir will increase the level or effect of loperamide by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.
- granisetron
granisetron and loperamide both increase QTc interval. Use Caution/Monitor.
- hydroxyzine
hydroxyzine and loperamide both increase QTc interval. Use Caution/Monitor.
- imlunestrant
imlunestrant increases levels of loperamide by P-glycoprotein (MDR1) efflux transporter. Modify Therapy/Monitor Closely. Monitor for loperamide adverse effects if coadministered with moderate P-gp inhibitors. Modify dose if needed. .
- istradefylline
istradefylline will increase the level or effect of loperamide by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor. Istradefylline 40 mg/day increased peak levels and AUC of P-gp substrates in clinical trials. Consider dose reduction of sensitive P-gp substrates.
- itraconazole
itraconazole and loperamide both increase QTc interval. Use Caution/Monitor.
- ivacaftor
ivacaftor increases levels of loperamide by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor. Ivacaftor and its M1 metabolite has the potential

to inhibit P-gp; may significantly increase systemic exposure to sensitive P-gp substrates with a narrow therapeutic index.

- ketoconazole
ketoconazole will increase the level or effect of loperamide by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.
- lapatinib
lapatinib will increase the level or effect of loperamide by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.
- lenacapavir
lenacapavir will increase the level or effect of loperamide by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Use Caution/Monitor. Lenacapavir may increase CYP3A4 substrates initiated within 9 months after last SC dose of lenacapavir, which may increase potential risk of adverse reactions of CYP3A4 substrates.
- levoketoconazole
levoketoconazole will increase the level or effect of loperamide by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.
- lithium
lithium and loperamide both increase QTc interval. Use Caution/Monitor.
- lonafarnib
lonafarnib will increase the level or effect of loperamide by affecting hepatic/intestinal enzyme CYP3A4 metabolism. Modify Therapy/Monitor Closely. Loperamide is contraindicated in patients aged <2 years; when lonafarnib is coadministered with loperamide, do not exceed loperamide 1 mg qDay when first coadministered; slowly increase loperamide dosage with caution in accordance with its approved product labeling.

lonafarnib will increase the level or effect of loperamide by P-glycoprotein (MDR1) efflux transporter. Modify Therapy/Monitor Closely. Lonafarnib is a weak P-gp inhibitor. Monitor for adverse reactions if coadministered with P-gp substrates where minimal concentration changes may lead to serious or life-threatening toxicities. Reduce P-gp substrate dose if needed.
- loratadine
loratadine will increase the level or effect of loperamide by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.
- lovastatin
lovastatin will increase the level or effect of loperamide by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.
- mavoxixafor
mavoxixafor and loperamide both increase QTc interval. Modify Therapy/Monitor Closely. Mavoxixafor causes concentration-dependent QTc prolongation. Monitor QTc during treatment in patients with risk factors for QTc prolongation (eg, coadministered medications that increase mavoxixafor exposure or other drugs with a high risk to prolong the QTc interval). Mavoxixafor dose reduction or

discontinuation may be required.

mavoxiafor will increase the level or effect of loperamide by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor. Caution if mavoxiafor (a P-gp inhibitor) is coadministered with a sensitive P-gp substrate where minimal substrate concentration changes may lead to serious adverse effects.

- metoclopramide intranasal
loperamide will decrease the level or effect of metoclopramide intranasal by Other (see comment). Use Caution/Monitor. Coadministration of metoclopramide intranasal with drugs that impair GI motility may decrease systemic absorption of metoclopramide. Monitor for reduced therapeutic effect.
- mirtazapine
mirtazapine and loperamide both increase QTc interval. Use Caution/Monitor.
- nefazodone
nefazodone will increase the level or effect of loperamide by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.
- nicardipine
nicardipine will increase the level or effect of loperamide by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.
- nifedipine
nifedipine will decrease the level or effect of loperamide by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.
- nilotinib
nilotinib will increase the level or effect of loperamide by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.
- olanzapine
olanzapine and loperamide both increase QTc interval. Use Caution/Monitor.
- osilodrostat
osilodrostat and loperamide both increase QTc interval. Use Caution/Monitor.
- phenobarbital
phenobarbital will decrease the level or effect of loperamide by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.
- phenytoin
phenytoin will decrease the level or effect of loperamide by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.
- ponatinib
ponatinib increases levels of loperamide by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.
- primaquine
primaquine and loperamide both increase QTc interval. Use Caution/Monitor.

- quercetin
quercetin will decrease the level or effect of loperamide by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.
- ranolazine
ranolazine will increase the level or effect of loperamide by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.
- remibrutinib
remibrutinib increases levels of loperamide by P-glycoprotein (MDR1) efflux transporter. Modify Therapy/Monitor Closely. Monitor for loperamide adverse effects if coadministered with moderate P-gp inhibitors. Modify dose if needed. .
- revumenib
revumenib and loperamide both increase QTc interval. Use Caution/Monitor. ECG monitoring recommended if concomitant use can't be avoided and withhold therapy if QTc interval is >480 msec; restart therapy after QTc interval returns to <480 msec
- rifampin
rifampin will decrease the level or effect of loperamide by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.
- ritonavir
ritonavir will increase the level or effect of loperamide by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.
- sarecycline
sarecycline will increase the level or effect of loperamide by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor. Monitor for toxicities of P-gp substrates that may require dosage reduction when coadministered with P-gp inhibitors.
- sertraline
sertraline and loperamide both increase QTc interval. Use Caution/Monitor.
- sevabertinib
sevabertinib increases levels of loperamide by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor. Sevabertinib is a weak-to-moderate P-gp inhibitor. Monitor sensitive P-gp substrates where minimal increases in concentration may lead to serious adverse reactions.
- simvastatin
simvastatin will increase the level or effect of loperamide by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.
- siponimod
siponimod and loperamide both increase QTc interval. Use Caution/Monitor.
- sirolimus
sirolimus will increase the level or effect of loperamide by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.
- solifenacin

solifenacin and loperamide both increase QTc interval. Use Caution/Monitor.

- spironolactone
spironolactone will increase the level or effect of loperamide by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.
- St John's Wort
St John's Wort will decrease the level or effect of loperamide by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.
- stiripentol
stiripentol will increase the level or effect of loperamide by P-glycoprotein (MDR1) efflux transporter. Modify Therapy/Monitor Closely. Consider reducing the dose of P-glycoprotein (P-gp) substrates, if adverse reactions are experienced when administered concomitantly with stiripentol.
- sunitinib
sunitinib and loperamide both increase QTc interval. Use Caution/Monitor.
- sunvozertinib
sunvozertinib increases levels of loperamide by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor. Follow substrate recommendations.
- tacrolimus
tacrolimus will increase the level or effect of loperamide by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.

tacrolimus and loperamide both increase QTc interval. Use Caution/Monitor.
- taletrectinib
taletrectinib and loperamide both increase QTc interval. Use Caution/Monitor. Concomitant use may increase the risk of QTc interval prolongation. If concomitant use is unavoidable, adjust the frequency of ECG and electrolyte monitoring.
- tolvaptan
tolvaptan will increase the level or effect of loperamide by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.
- trazodone
trazodone will decrease the level or effect of loperamide by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.
- tucatinib
tucatinib will increase the level or effect of loperamide by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor. Consider reducing the dosage of P-gp substrates, where minimal concentration changes may lead to serious or life-threatening toxicities.
- valbenazine
valbenazine and loperamide both increase QTc interval. Use Caution/Monitor.
- vemurafenib

vemurafenib increases levels of loperamide by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.

- verapamil
verapamil will increase the level or effect of loperamide by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor.
- vimseltinib
vimseltinib increases levels of loperamide by P-glycoprotein (MDR1) efflux transporter. Use Caution/Monitor. If coadministration cannot be avoided, take the drug 4 hours before taking the P-gp substrate; increases risk of QT prolongation, CNS depression, cardiac arrhythmias.
- vorinostat
vorinostat and loperamide both increase QTc interval. Use Caution/Monitor.
- ziftomenib
loperamide, ziftomenib. QTc interval. Use Caution/Monitor. Caution if ziftomenib is coadministered with drugs that exacerbated risk of QT prolongation (eg, hypokalemia, orthostatic hypotension, inhibits metabolism of ziftomenib).

Minor (2)

- gemfibrozil
gemfibrozil will increase the level or effect of loperamide by decreasing metabolism. Minor/Significance Unknown.
- itraconazole
itraconazole will increase the level or effect of loperamide by Other (see comment). Minor/Significance Unknown. Monitor ECG when itraconazole is coadministered with loperamide (dose >16mg/day).

Adverse Effects

Frequency Not Defined

Dizziness

Fatigue

Abdominal pain

Constipation

Nausea

Dry mouth

Angioedema

Bullous eruptions

Flatulence

Rash

Postmarketing Reports

Pancreatitis

QT/QTc interval prolongation, Torsades de Pointes, other ventricular arrhythmias, cardiac arrest, syncope, and death

Warnings

Black Box Warnings

Torsades de Pointes and sudden death

- Cases of Torsades de Pointes, cardiac arrest, and death reported with use of higher than recommended dosages
- Contraindicated in patients < 2 years
- Avoid dosages higher than recommended in adults and pediatric patients 2 years of age and older due to risk of serious cardiac adverse reactions

Contraindications

Hypersensitivity, bloody diarrhea, high fever, infectious diarrhea, pseudomembranous colitis

Patients in whom constipation must be avoided

Abdominal pain without diarrhea

Avoid use as primary therapy with acute dysentery (bloody stools and high fever, acute ulcerative colitis, bacterial enterocolitis [caused by *Salmonella*, *Shigella*, and *Campylobacter*], pseudomembranous colitis associated with antibiotic use)

Age <2 years

Cautions

May cause drowsiness or dizziness, which may impair physical abilities to operate heavy machinery or tasks requiring mental alertness

Hypersensitivity reactions reported, including anaphylaxis, rash, urticaria, and rare cases of Steven's Johnson syndrome or toxic epidermal necrolysis

Discontinue if no improvement seen within 48 hours in patients with acute diarrhea, symptoms worsen, or abdominal swelling or bulging develops

Discontinue promptly if constipation, abdominal pain or distention, blood in stool, or ileus develops; do not use when peristalsis inhibition should be avoided (ie, due to potential for ileus, megacolon, or toxic megacolon)

Discontinue therapy if symptoms of abdominal distention occur in patients with AIDS; cases of toxic megacolon reported with infectious colitis, resulting from viral or bacterial pathogens

Use with caution in patients with hepatic impairment due to reduced first pass metabolism; monitor for signs of CNS toxicity

Use of higher than recommended doses or abuse of loperamide can result in serious cardiac adverse events, including QT interval prolongation, Torsades de Pointes, or other ventricular arrhythmias, syncope, and cardiac arrest; in cases of abuse, individuals often use other drugs together with loperamide in attempts to increase its absorption and penetration across the blood-brain barrier, inhibit loperamide metabolism, and enhance its euphoric effects

Dehydration, particularly in pediatric patients less than 6 years of age, may further influence variability of response to loperamide

Avoid loperamide in combination with drugs or herbal products known to prolong QT interval, including Class 1A (e.g., quinidine, procainamide) or Class III (e.g., amiodarone, sotalol) antiarrhythmics, antipsychotics (e.g., chlorpromazine, haloperidol, thioridazine, ziprasidone), antibiotics (e.g., moxifloxacin), or any other drug known to prolong the QT interval (e.g., pentamidine, methadone)

Avoid administering therapy to patients with risk factors for QT prolongation, including patients with congenital long QT syndrome, with a history of cardiac arrhythmias or other cardiac conditions, elderly patients and those with electrolyte abnormalities

Patients with a history of abnormal heart rhythm or liver disease, should ask their healthcare provider before using the product; taking more than directed can cause serious heart problems or death

Potential toxic dose in patients <6 years: 0.4 mg/kg

Pregnancy & Lactation

Pregnancy category: B

Lactation: Not known if distributed in breast milk; use caution

Pregnancy Categories

A: Generally acceptable. Controlled studies in pregnant women show no evidence of fetal risk.

B: May be acceptable. Either animal studies show no risk but human studies not available or animal studies showed minor risks and human studies done and showed no risk.

C: Use with caution if benefits outweigh risks. Animal studies show risk and human studies not available or neither animal nor human studies done.

D: Use in LIFE-THREATENING emergencies when no safer drug available. Positive evidence of human fetal risk.

X: Do not use in pregnancy. Risks involved outweigh potential benefits. Safer alternatives exist.

NA: Information not available.

Pharmacology

Mechanism of Action

Slows intestinal motility through opioid receptor; has direct effects on circular and longitudinal muscle; reduces fecal volume; increases viscosity

Absorption

Bioavailability: 0.3%

Onset: 1-3 hr

Duration: 41 hr

Peak plasma time: 5 hr (capsule); 2.5 hr (liquid)

Distribution

Poor penetration through blood-brain barrier

Metabolism

Significant first-pass metabolism, resulting in very low plasma level of drug

Metabolites: Glucuronide (inactive)

Elimination

Elimination half-life: 7-14 hr

Excretion: Feces (30-40%), urine (1%)

Formulary

Adding plans allows you to compare formulary status to other drugs in the same class.

Formulary information is available for health plans only in the United States.

Medscape prescription drug monographs are based on FDA-approved labeling information, unless otherwise noted, combined with additional data derived from primary medical literature.
